جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : خضره عبدالمنعم عبدالوهاب القصاص
المحافظة : الجيزه **الحي / المدينة :** بولاق الدكرور **العنوان :** ..
رقم البطاقة : 26806302101143 **رقم التليفون :** 01126796311 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 30-06-1968 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: خضره عبدالمنعم عبدالوهاب القصاص
National ID: 26806302101143

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

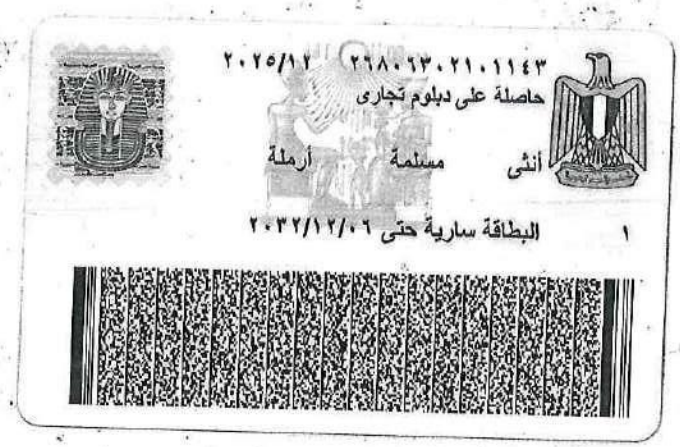
نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Bilateral mammogram and breast ultrasound**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





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٠١١١٤٤٠١٥٦٩

National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

05/24/2026

MDT FORM

Name:	مصرة عبدالله عبدالوهاب القصاص	**MRN:**	9866
Age:	58 Y	**ID:**	26806302101143
Menopausal Sta	post	**Phone:**	01126796311
NRS SCORE:		**Marital Status:**	married
BMI:		**Family History:**	-ve
CO Morbidity:	free	**Reason Of MDT:**	new

Clinical Data:

BX: IDC g2

ER: 8/8 **PR:** 6/8 **HER2:** 1+ **KI67:** 35%

Sonomammography:
rt UOQ susp mass 2.8x4.5x 4.5 cm + indeterminate axillary LNs (ct2 n1

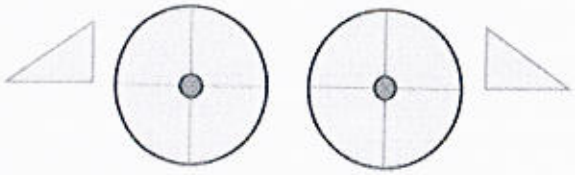
Mets Workup:
ct:
-Right breast neoplastic mass with associated skin thickening suggestive of locally advanced/ inflammatory breast cancer for sono-mammographic correlation and histopathology.
-Multiple right axillary lymph nodes? Metastatic. Hepatomegaly .
-No CT evidence of distant metastatic disease.

History:
clinically: rt LOQ mass with overlying skin puckering + palpable axillary LNs
for possibility of neoadj tlt then of BCS if responding
clinical photos are uploaded
omnia essawy

Recommendations:

MDT Outcome & Plan
for NACT AC 4 TAXOTERE 4 after clip insertion
BCS ALND

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr Hanan Ezzat	DR AHMED NAGIB				



MDT Form
[[DOCTYPE:MDT_FORM]]

Name: خضره عبدالمنعم عبدالوهاب القصاص **MR. :** 9866	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : | **تاريخ وساعة الخروج :**

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient underwent a multidisciplinary team (MDT) discussion in May 2026, during which the decision was made to initiate neoadjuvant chemotherapy (NACT) with AC (doxorubicin 100 mg/m² and cyclophosphamide 1000 mg/m²) for four cycles followed by taxotere for four cycles after clip insertion and breast-conserving surgery (BCS) with axillary lymph node dissection (ALND).

In June 2026, the patient was diagnosed with right breast cancer (clinical stage II, T2 N1 M0) with histology IDC, ER 8/8, PR 6/8, HER2-negative, Ki-67 35%. Imaging revealed a 28 × 4.5 × 4.5 mm mass in the right upper outer quadrant (RUQ) and indeterminate axillary lymph nodes. Physical examination confirmed a 4 × 5 cm RUQ mass. The MDT recommended 4 cycles of AC followed by BCS, with clip placement for future localization. The patient received the first cycle of AC (doxorubicin 100 mg/m², cyclophosphamide 1000 mg/m²) and endoxan 1000 mg, with echocardiography showing a left ventricular ejection fraction of 69 % and weight 69 kg, height 159 cm, BMI 1.74. The patient was also started on adjuvant adria 100 mg and endoxan 1000 mg.

On 16 June 2026, the patient completed the first cycle of AC with adria 100 mg and endoxan 1000 mg. Echocardiography remained stable at 63 % ejection fraction. Laboratory values from the previous week were acceptable.

On 7 July 2026, the patient presented with a 3 × 3 cm mass in the right lower outer quadrant (RLOQ) with skin dimpling. The patient received the second cycle of AC (adria 100 mg, endoxan 1000 mg) and had neoadjuvant clips inserted for future localization.

On 28 July 2026, the patient received the third cycle of AC (adria 100 mg, endoxan 1000 mg). The RLOQ mass measured 2 × 2 cm or less. The patient experienced neutropenic fever after the first and second cycles and was given prophylactic neubogen twice.

On 18 August 2026, the patient received the fourth cycle of AC. Laboratory results from three days prior were acceptable. The patient was prescribed methabiogen and scheduled for the next visit.

On 8 September 2026, the patient was examined and found to have a 1 × 2 cm mass in the RLOQ. The patient began the first cycle of taxotere neoadjuvant therapy at 110 mg (10 % dose reduction) and was assessed for tolerability. The patient's weight was 60 kg, height 159 cm, BMI 1.6. The patient has completed 4 cycles of AC and 1 cycle of taxotere to date.

تحريرا في ,

التاريخ : 20-09-2026 **الوقت :**

الطبيب المعالج : dina makram

يعتمد ,

STAVE/
USSY
TIONAL

atient Name	: الأستاذه / خضره عبد المنعم عبد الوهاب القصاص	Request No.	: 13005487
atient No.	: 26806302101143	Request Date	: 17/03/2026
th No.	: 735/26	Verification Date	: 28/03/2026
ex	: Female	Financial Category	: جوستاف روسي - مبادره
ge	: 57 Y	Department	: Out Patient
eferreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

linical Data: Right breast UOQ mass.
ature of Specimen: Tru-cut biopsy, collected on 15/3/2026, at 1:15 pm. Cold ischemia time 20 seconds.

ross Examination:
our grey white tissue cores, ranging in size between 0.2 cm & 1.3 cm long, totally ubmitted.

icroscopic Examination:
Histological type: Invasive duct carcinoma.
Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).
Ductal carcinoma in situ: Absent.
Perineural invasion: Absent.
Angioinvasion:Absent.
Microcalcification: Absent.

iagnosis:
ight breast mass, Tru-cut biopsy:
nvasive duct carcinoma, Grade 2.
For further IHC (ER, PR, Her2, Ki67).

ignature
r. Walaa Ibrahim Ghonaim
r. Heba El Shafeey.

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

ient Name	: الأستاذه / خضره عبد المنعم عبد الوهاب القصاص	Request No.	: 13005487
ient No.	: 26806302101143	Request Date	: 17/03/2026
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‹	: Female	Financial Category	: جوستاف روسي - مبادره
ɜ	: 57 Y	Department	: Out Patient
erreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

cal Data: Right breast UOQ mass, diagnosed "Invasive duct carcinoma grade 2".
ıre of Specimen: Tru-cut biopsy, collected on 15/3/2026, at 1:15 pm. Cold ischemia time is 20 ɔnds.

ınique:
ctions prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

ılts:
(All controls showed appropriate reactivity):
: Strong nuclear reaction in 95% of tumor cells (score 8/8).
: Strong nuclear reaction in 30% of tumor cells (score 6/8).
r2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
7%: 35%.

clusion:
t breast mass, Tru-cut biopsy:
asive duct carcinoma, Grade 2.

: Positive (score 8/8).
: Positive (score 6/8).
-2: Low (score 1+).
7%: 35%.

ɔring system: "ER, PR"
O-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, ; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

Verified by :
Dr.Heba Hasan

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غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

ient Name	الأستاذه / خضره عبد المنعم عبد الوهاب القصاص	Request No.	13005487
ient No.	26806302101143	Request Date	17/03/2026
ı No.	735/26	Verification Date	28/03/2026
	Female	Financial Category	جوستاف روسي - مبادره
	57 Y	Department	Out Patient
erreing Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, mediate; and 3, strong). Total score from 0 to 8.
ɔring system: "Her2"
rence: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining rved. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely ɐptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane ing that is faint/barely perceptible and within > 10% of the invasive tumor cells.
2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and ı >10% of
ıvasive tumor cells or complete and circumferential membrane staining that is intense and within < of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, se > 10% of tumor cells.

ature
/alaa Ibrahim Ghonaim
eba El Shafeey

Verified by :
Dr.Heba Hasan

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غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Name : KHADRA ABDELMONEIM ABDELWAHAB	Date : 23-02-2026 12:20
Branch : Portable	Medical No. : GRI-230226121618P
Ref.Physician Prof :	Ins.Company :

BILATERAL DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH :RESOLUTION BREAST SONOGRAPHY REVEALED

CLINICAL SUMMARY:
RIGHT breast palpable lump

FINDINGS:

RIGHT BREAST:

- Scattered fibroglandular density , (ACR b).
- UOQ speculated dense mass
- Ultrasound demonstrated an irregular hypoechoic speculated mass at 10 oclock, 7 cm from the nipple, measuring 2.8 x 4.5 x 4.5 cm , with marked internal vascularity
- A level I axillary node is seen with cortical thickness reaching 4.7 mm, indeterminate.
- No suspicious supraclavicular or infraclavicular lymph nodes

Left BREAST:

- Scattered fibroglandular density , (ACR b).
- No mass, spiculation or suspicious micro calcifications, this was further confirmed by and U/S examination which depicted mottled parenchyma.
- Normal skin/nipple/areola complex.
- Non specific axillary nodes are noted.

CONCLUSION:
Right breast (BIRADS 4c): UOQ suspicious mass as described for histopathological correlation
Left breast (BIRADS 1): normal

Dr farah elshobaky

Mammogram
[[DOCTYPE:MAMMOGRAM]]